



Division of Provider Services
& Quality Assurance
P.O. Box 8059, Slot S404
Little Rock, AR 72203-8059

July 15, 2025

David Napier, Administrator
Youth Home Inc
20400 Colonel Glenn Road
Little Rock, AR 72210-5323

Dear Mr. Napier:

On July 3, 2025 a Complaint Investigation survey was conducted at your facility by the Office of Long Term Care to determine if your facility was in compliance with Federal requirements for Psychiatric Residential Treatment Facilities participating in the Medicaid (Title XIX) Program. This survey found that your facility had deficiencies requiring correction/substantial correction prior to a revisit as specified in the attached CMS-2567.

Plan of Correction

A POC must be submitted within 10 calendar days of you receipt of the Statement of Deficiencies. Failure to submit a POC may result in termination. Include a completion date for each deficiency cited.

Stephanie Lester, Reviewer
OLTC, Survey & Certification Section
PO Box 8059, Slot S404
Little Rock, AR 72201-4608
(501) 396-6013
email to stephanie.lester@dhs.arkansas.gov.

Your Plan of Correction must also include the following:

- a. Address how the corrective action will be accomplished for those residents found to have been affected by the deficient practice;
- b. Address how the facility will identify other residents having the potential to be affected by the same deficient practice;
- c. Address what measures will be put into place or systemic changes will be made to ensure that the deficient practice will not recur;
- d. Indicate how the facility plans to monitor its performance to make sure that solutions are sustained. The facility must develop plan for ensuring that correction is achieved and sustained. This plan must be implemented, and the corrective action evaluated for its effectiveness.

e. Include dates when corrective action will be completed. The corrective action completion dates must be acceptable to the State. Your facility is ultimately accountable for its own compliance. The plan of correction will serve as the facility's allegation of compliance. Unless otherwise stated on the PoC, the last completion date will be the date of alleged compliance.

Informal Dispute Resolution

In accordance with 42 CFR § 488.331, you have one opportunity to question deficiencies through an informal dispute resolution (IDR) process. To obtain an IDR, you must send your written request to Health Facility Services, Arkansas Department of Health within ten (10) calendar days from receipt of the Statement of Deficiencies. The request must state the specific deficiencies the facility wishes to challenge. The request should also state whether the facility wants the IDR to be performed by a telephone conference call, record review, or a face-to-face meeting.

If the facility will be represented by an Attorney, please notify ADH in your IDR request and provide the name and contact number for the Attorney. Additionally, please notify DHS, at OLTC2@dhs.arkansas.gov, of your intent to be represented by an Attorney. Failure to do so may result in a delay of your hearing.

An incomplete informal dispute resolution procedure will not delay the effective date of any enforcement action or the requirement for timely submission of an acceptable plan of correction. Informal dispute resolution in no way is to be construed as a formal evidentiary hearing. It is an informal administrative process to discuss the findings.

Please submit your request to:

**IDR/IIDR Program Coordinator
Health Facilities Services
5800 West 10th Street, Suite 400
Little Rock, AR 72204
Phone: 501-661-2201
ADH.HFS@Arkansas.gov**

If you have any questions, please contact your Reviewer.

Sincerely,



for Jeff Rosenbaum

Stephanie Lester, RN QA Reviewer
DPSQA/Office of Long Term Care
Survey & Certification Section

sl

cc: DRA

DEPARTMENT OF HEALTH AND HUMAN SERVICES
CENTERS FOR MEDICARE & MEDICAID SERVICES

PRINTED: 07/15/2025
FORM APPROVED
OMB NO. 0938-0391

STATEMENT OF DEFICIENCIES AND PLAN OF CORRECTION		(X1) PROVIDER/SUPPLIER/CLIA IDENTIFICATION NUMBER: 04L107	(X2) MULTIPLE CONSTRUCTION A. BUILDING _____ B. WING _____		(X3) DATE SURVEY COMPLETED C 07/03/2025
NAME OF PROVIDER OR SUPPLIER YOUTH HOME INC			STREET ADDRESS, CITY, STATE, ZIP CODE 20400 COLONEL GLENN ROAD LITTLE ROCK, AR 72210		
(X4) ID PREFIX TAG	SUMMARY STATEMENT OF DEFICIENCIES (EACH DEFICIENCY MUST BE PRECEDED BY FULL REGULATORY OR LSC IDENTIFYING INFORMATION)	ID PREFIX TAG	PROVIDER'S PLAN OF CORRECTION (EACH CORRECTIVE ACTION SHOULD BE CROSS-REFERENCED TO THE APPROPRIATE DEFICIENCY)	(X5) COMPLETION DATE	
N 000	Initial Comments Note: The CMS-2567 (Statement of Deficiencies) is an official, legal document. All information must remain unchanged except for entering the plan of correction, correction dates, and the signature space. Any discrepancy in the original deficiency citation(s) will be reported to the Dallas Regional Office (RO) for referral to the Office of the Inspector General (OIG) for possible fraud. If information is inadvertently changed by the provider/supplier, the State Survey Agency (SA) should be notified immediately. Complaint# NA (AR00039067) was not in compliance, with deficiencies cited at N143 and N156 The findings on this statement of deficiencies demonstrate the facility is not in compliance with the requirements of 42 CFR Part 483, Subpart B, for Long Term Care Facilities.	N 000			
N 143	ORDERS FOR USE OF RESTRAINT OR SECLUSION CFR(s): 483.358(d) If the order for restraint or seclusion is verbal, the verbal order must be received by a registered nurse or other licensed staff such as a licensed practical nurse, while the emergency safety intervention is being initiated by staff or immediately after the emergency safety situation ends. The physician or other licensed practitioner permitted by the state and the facility to order restraint or seclusion must verify the verbal order in a signed written form in the resident's record. The physician or other licensed practitioner permitted by the state and the facility to order restraint or seclusion must be available to staff for	N 143			

LABORATORY DIRECTOR'S OR PROVIDER/SUPPLIER REPRESENTATIVE'S SIGNATURE

TITLE

(X6) DATE

Any deficiency statement ending with an asterisk (*) denotes a deficiency which the institution may be excused from correcting providing it is determined that other safeguards provide sufficient protection to the patients. (See instructions.) Except for nursing homes, the findings stated above are disclosable 90 days following the date of survey whether or not a plan of correction is provided. For nursing homes, the above findings and plans of correction are disclosable 14 days following the date these documents are made available to the facility. If deficiencies are cited, an approved plan of correction is requisite to continued program participation.

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N 143	Continued From page 1 consultation, at least by telephone, throughout the period of the emergency safety intervention. This ELEMENT is not met as evidenced by: Based on record review, interviews, and facility policy review, the facility failed to ensure Physician ' s Orders were obtained and documented for personal restraints for one (Client #1) of two clients reviewed for having received personal restraints. The findings include: Review of Client #1's Face Sheet, indicated Client #1 had diagnoses which included a disorder which caused moderate inability to form appropriate attachments, trauma and stressor disorder, generalized anxiety disorder, and attention deficit hyperactivity disorder. Review of an Incident Report (IR) for Client #1 indicated the client was placed in a personal restraint on the following dates and times: 04/29/2025 from 5:27 PM until 5:28 PM, and 05/07/2025 from 12:48 PM until 12:49 PM. Review of Physician ' s Orders dated 04/29/2025, and 05/07/2025, revealed no orders were documented for personal restraints on those dates. On 07/02/2025 at 11:59 AM, during an interview the Assistant Director of Nursing (ADON) verbally confirmed that orders were required for all personal restraints. The ADON checked through Client #1 ' s IRs and Physician ' s Orders for the personal restraint documentation. The ADON then verified the restraints were implemented, but	N 143			

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N 143	Continued From page 2 there were no Physician ' s Orders documented on 04/29/2025 or 05/07/2025. Review of a facility policy titled " Personal Restraint-Intensive Residential, " with a last review date of 07/24/2023, indicated that an order is to be obtained as soon as possible after initiation of a personal restraint, and must be signed by the ordering provider as soon as possible, but within 48 hours of the intervention.	N 143			
N 156	ORDERS FOR USE OF RESTRAINT OR SECLUSION CFR(s): 483.358(j) The physician or other licensed practitioner permitted by the state and the facility to order restraint or seclusion must sign the restraint or seclusion order in the resident's record as soon as possible. This ELEMENT is not met as evidenced by: Based on record review, interviews, and facility policy review, the facility failed to ensure physician orders were signed within 48 hours of implementation of personal restraints for two (Client #1 and Client #3) of two clients reviewed for having received personal restraints. The findings include: Client #1 Review of Client #1's Face Sheet, dated 07/01/2025, indicated Client #1 had diagnoses which included a disorder which caused moderate inability to form appropriate attachments, trauma and stressor disorder, generalized anxiety disorder and attention deficit	N 156			

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N 156	Continued From page 3 hyperactivity disorder (ADHD). A review of a Physician Order, dated 05/09/2025, indicated an order for a personal restraint was received for Client #1 at 4:33 PM, and was signed by Advanced Practice Registered Nurse (APRN) #1 on 05/12/2025 at 8:54 AM A review of a Physician Order dated 05/29/2025, indicated an order for a personal restraint was received for Client #1 at 5:38 PM and was signed by APRN #1 on 06/02/2025 at 5:40 PM. A review of a Physician Order dated 06/06/2025, indicated an order for a personal restraint was received for Client #1 at 7:07 PM, and was signed by Psychiatrist and/or Medical Doctor on 06/16/2025 at 6:33 PM. A review of a Physician Order, dated 06/12/2025, indicated an order for a personal restraint was received for Client #1 at 4:14 PM, and was signed by APRN #1 on 06/16/2025 at 9:30 AM Client #3 A review of Client #3's Face Sheet, dated 07/02/2025, indicated Client #3 had diagnoses which included diagnoses of trauma and stressor related disorder, and ADHD. A review of a Physician Order, dated 05/30/2025, indicated an order for a personal restraint was received for Client #3 at 1:34 PM, and was signed by Psychiatrist and/or Medical Doctor on 06/04/2025 at 8:32 AM A review of a Physician Order, dated 06/12/2025, indicated an order for a personal restraint was received for Client #3 at 2:59 PM, and was signed	N 156			

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N 156	Continued From page 4 by APRN #1 on 06/16/2025 at 9:31 AM On 07/02/2025 at 11:59 AM, during an interview, the Assistant Director of Nursing (ADON) verbally confirmed that orders were required for all personal restraints. The ADON reviewed Client #1 's Incident Reports and Physician 's Orders for the personal restraints and verified the restraints were implemented, but the physician orders for 05/09/2025 and 06/06/2025 were not signed within 48 hours. On 07/02/2025 at 12:26 PM during an interview, the Director of Nursing (DON) stated that if a provider issued a restraint order after leaving the facility, the order would typically be signed when the provider returned to the facility on their next scheduled workday. The DON stated they would address the issue and work on changing the policy to ensure the orders were signed within the required time frame. On 07/02/2025 at 2:18 PM during an interview, Advanced Practice Registered Nurse (APRN) #1 reported he typically signed telephone orders given after he left the facility when he returned from scheduled days off. APRN #1 reported he was currently working on a plan to ensure orders were signed within the required time frame. Review of a facility policy titled Personal Restraint-Intensive Residential last review date of 07/24/2023, indicated that an order is to be obtained as soon as possible after initiation of a personal restraint and must be signed by the ordering provider as soon as possible, but within 48 hours of the intervention.	N 156			

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LABORATORY DIRECTOR'S OR PROVIDER/SUPPLIER REPRESENTATIVE'S SIGNATURE

TITLE

(X6) DATE

Beverly Foti

Chief Regulatory Officer

7/24/25

Any deficiency statement ending with an asterisk (*) denotes a deficiency which the institution may be excused from correcting providing it is determined that other safeguards provide sufficient protection to the patients. (See instructions.) Except for nursing homes, the findings stated above are disclosable 90 days following the date of survey whether or not a plan of correction is provided. For nursing homes, the above findings and plans of correction are disclosable 14 days following the date these documents are made available to the facility. If deficiencies are cited, an approved plan of correction is requisite to continued program participation.

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N 156	Continued From page 3 hyperactivity disorder (ADHD). A review of a Physician Order, dated 05/09/2025, indicated an order for a personal restraint was received for Client #1 at 4:33 PM, and was signed by Advanced Practice Registered Nurse (APRN) #1 on 05/12/2025 at 8:54 AM A review of a Physician Order dated 05/29/2025, indicated an order for a personal restraint was received for Client #1 at 5:38 PM and was signed by APRN #1 on 06/02/2025 at 5:40 PM. A review of a Physician Order dated 06/06/2025, indicated an order for a personal restraint was received for Client #1 at 7:07 PM, and was signed by Psychiatrist and/or Medical Doctor on 06/16/2025 at 6:33 PM. A review of a Physician Order, dated 06/12/2025, indicated an order for a personal restraint was received for Client #1 at 4:14 PM, and was signed by APRN #1 on 06/16/2025 at 9:30 AM Client #3 A review of Client #3's Face Sheet, dated 07/02/2025, indicated Client #3 had diagnoses which included diagnoses of trauma and stressor related disorder, and ADHD. A review of a Physician Order, dated 05/30/2025, indicated an order for a personal restraint was received for Client #3 at 1:34 PM, and was signed by Psychiatrist and/or Medical Doctor on 06/04/2025 at 8:32 AM A review of a Physician Order, dated 06/12/2025, indicated an order for a personal restraint was received for Client #3 at 2:59 PM, and was signed	N 156	This will occur each day until the order is signed. The log with deficient signatures will be kept in the Records Compliance share-point folder for future reference. 3. The Chief Medical Officer will send out the process to all providers who would be giving restraint orders. Contained in that communication will be a reminder that orders for restraint must be signed within 48 hours. That means that if an order is given on a Friday, Saturday, or Sunday, they must still ensure signature within 48 hours. That communication will go out from the Chief Medical Officer on 07/25/25. 4. Records Compliance Director will instruct departmental staff on the process to be followed. This process was explained verbally to staff on 07/22/25 and followed with written instructions on 07/23/25. 5. To monitor performance, orders for incidents will be checked daily Monday-Friday in perpetuity; the metric for results of this process will be added to the agenda for the Performance Improvement Committee that meets quarterly. Members of this committee include the Chief Executive Officer, the Chief Medical Officer, Chief Clinical Officer, Chief Regulatory Officer, and all departmental directors.		

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NAME OF PROVIDER OR SUPPLIER YOUTH HOME INC			STREET ADDRESS, CITY, STATE, ZIP CODE 20400 COLONEL GLENN ROAD LITTLE ROCK, AR 72210		
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N 156	Continued From page 4 by APRN #1 on 06/16/2025 at 9:31 AM On 07/02/2025 at 11:59 AM, during an interview, the Assistant Director of Nursing (ADON) verbally confirmed that orders were required for all personal restraints. The ADON reviewed Client #1 ' s Incident Reports and Physician ' s Orders for the personal restraints and verified the restraints were implemented, but the physician orders for 05/09/2025 and 06/06/2025 were not signed within 48 hours. On 07/02/2025 at 12:26 PM during an interview, the Director of Nursing (DON) stated that if a provider issued a restraint order after leaving the facility, the order would typically be signed when the provider returned to the facility on their next scheduled workday. The DON stated they would address the issue and work on changing the policy to ensure the orders were signed within the required time frame. On 07/02/2025 at 2:18 PM during an interview, Advanced Practice Registered Nurse (APRN) #1 reported he typically signed telephone orders given after he left the facility when he returned from scheduled days off. APRN #1 reported he was currently working on a plan to ensure orders were signed within the required time frame. Review of a facility policy titled Personal Restraint-Intensive Residential last review date of 07/24/2023, indicated that an order is to be obtained as soon as possible after initiation of a personal restraint and must be signed by the ordering provider as soon as possible, but within 48 hours of the intervention.	N 156			



Division of Provider Services
& Quality Assurance
P.O. Box 8059, Slot S404
Little Rock, AR 72203-8059

July 25, 2025

David Napier, Administrator
Youth Home Inc
20400 Colonel Glenn Road
Little Rock, AR 72210-5323

Dear Mr. Napier:

On July 3, 2025, we conducted a Complaint Investigation survey at your facility. You have alleged that the deficiencies cited on that survey have been corrected. We are accepting your allegation of compliance and have approved your plan of correction and presume that you will achieve substantial correction by July 28, 2025.

We will be conducting a revisit of your facility to verify that substantial correction has been achieved and maintained.

If you have any questions, please contact your reviewer: **Stephanie Lester at 501-396-6013 or email to: stephanie.lester@dhs.arkansas.gov.**

Sincerely,

A handwritten signature in dark ink that reads "Stephanie Lester RN". The signature is written in a cursive, flowing style.

for Jeff Rosenbaum

Stephanie Lester, RN QA Reviewer
DPSQA/Office of Long Term Care
Survey & Certification Section

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Division of Provider Services
& Quality Assurance
P.O. Box 8059, Slot S404
Little Rock, AR 72203-8059

July 28, 2025

David Napier, Administrator
Youth Home Inc
20400 Colonel Glenn Road
Little Rock, AR 72210-5323

Dear Administrator:

During the Complaint Survey conducted on July 03, 2025 your facility was found to be in compliance with program requirements.

If you have any questions, please contact your reviewer: **Stephanie Lester at 501-396-6013 or email to stephanie.lester@dhs.arkansas.gov.**

Sincerely,

A handwritten signature in cursive script that reads "Stephanie Lester RN". The signature is written in dark ink on a light-colored, slightly textured background.

for Jeff Rosenbaum

Stephanie Lester, RN QA Reviewer
DPSQA/Office of Long Term Care
Survey and Certification Section

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